Assignment 1

Community Drug Use Patterns in Pakistan

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Introduction

In Pakistan, drug abuse is a serious and expanding psychological and public health issue that impacts millions of people, their families, and larger communities. It is influenced by Pakistan's strategic location along important international drug trafficking routes and the nation's socioeconomic vulnerabilities. Individuals, especially young people, are more likely to experiment with and rely on psychoactive substances as coping mechanisms due to internal factors such as widespread poverty, unemployment, limited educational opportunities, rapid urbanization, population displacement, and inadequate access to mental health services. Lack of jobs, inadequate education, and being in a poor bracket are major factors that lead to drug abuse among the youth from needy backgrounds in Pakistan (Zaman et al., 2024). Substance use is further increased by psychological stressors like insecurity, exposure to violence, family conflicts, and social marginalization, which frequently turn drugs into a way to control emotions, escape, or feel like they belong. Drug use is more prevalent among individuals who lack adequate emotional support from caregivers or come from families with a history of substance use (Farrell & White, 1988). Evidence further suggests that young people are particularly vulnerable in dysfunctional family environments marked by poor communication, parental conflict, and insufficient supervision, factors that increase the likelihood of substance use initiation and continuation (Kilpatrick et al., 2000).

Through fieldwork, many patients in Greenfield Hospital of Psychiatry, located in Bani Gala, where brought because of psychiatric illnesses due to drug use. Most of them were young adults, aged 18 to 35. Use of drug is mostly attributed to psychological stressors as found in Greenfield Hospital of Psychiatry, but that can be associated more to the individual

who can access this type of private rehabilitation. From a public health perspective, these factors put a significant strain on already underfunded healthcare systems by causing early initiation, persistent dependence, and high relapse rates.

Literature Review of Drug Use Trends

In 2012 and 2013, United Nations Office on Drugs and Crime (UNODC) in collaboration with the Ministry of Narcotics Control, the U.S. Department of State's Bureau of International Narcotics and Law Enforcement Affairs (INL), conducted a national survey on drug use to estimate the extent and patterns of drug use in Pakistan. The overall results of the survey revealed that approximately 6% of the population, 9% of the adult male population and 2.9% of the adult female population, equivalent to 6.7 million people, had used a substance other than alcohol and tobacco in the preceding year (Yaqub, 2013).

In 2013, cannabis emerged as the most widely used illicit substance in Pakistan, with an estimated 4.03 million individuals (3.6%) aged 15–64 years reporting its use. The consumption of painkillers, affecting approximately 1.69 million people (1.5%), and opiates, used by around 1.02 million individuals (0.9%), including an estimated 813,000 heroin users (0.7%). The report further identified about 423,000 people (0.4%) as injecting drug users, with nearly 73% of regular opiate injectors reporting the sharing of syringes (UNODC, 2013)

As reported by United Nations Office on Drugs and Crime (UNODC) in 2022, National Drug Use Survey Pakistan 2022-24 was launched but the complete results have not been released publicly. It was conducted to assess the current situation in Pakistan, due to changing trends of drug use, mostly because of rapid increase in the use of synthetic drugs, such as Methamphetamine. Through the fieldwork done, as reported by Ms Marrium Mir, senior psychologist and psychotherapist in The Wellbeing House in Bani Gala, that a surge in cases of drug use is seen over the years, mostly attributed to amphetamine (ice) and heroin.

A major national survey was jointly carried out by the Anti-Narcotics Force (ANF) and the United Nations Office on Drugs and Crime (UNODC) estimated that Pakistan had approximately 9.6 million drug users in 2022, a large proportion of whom were between 15 and 35 years of age (ANF & UNODC, 2022). The findings indicated that cannabis and opioids continue to be the most commonly used illicit substances. Prevalence, however, varies across regions. For example, a study from Khyber Pakhtunkhwa reported a lifetime substance use rate of 32.5% among male university students, with cigarettes (21.5%), naswar (16.8%), and cannabis (10.5%) most frequently used (Khan et al., 2021). Similarly, another research conducted in Punjab showed that more than 47% of male medical students had used psychoactive substances, predominantly alcohol and cannabis (Ahmed et al., 2020).

Although substance use is considerably more prevalent among male youth, rates among females are widely regarded as substantially underreported due to strong social stigma and obstacles to disclosure, help-seeking, and survey participation (Khalid et al., 2019). As a result, substance use among young women often remains concealed, contributing to what has been described as a "silent epidemic." According to Drug Use in Pakistan 2013 Summary Report, high prevalence of non-medical prescription drug use has been observed nationwide, particularly among women. The vast majority of women who report drug use primarily misuse opioid-based painkillers, with a smaller proportion using tranquillizers and sedatives, substances that are easily accessible through pharmacies (UNODC, 2013).

Geographical Location

Pakistan's strategic geographic location along major drug trafficking corridors further intensifies the public health implications of substance abuse. Pakistan's proximity to Afghanistan, which produces over 80% of the world's opium, places Pakistan at the centre of the illicit drug transit network (Fazli, 2023). According to the Drug Use in Pakistan 2013

Summary Report, the highest levels of use of opiates, opium and heroin, are seen in the provinces (Balochistan, Khyber Pakhtunkhwa and FATA) which border principal poppy-cultivating areas in neighbouring Afghanistan (UNODC, 2013).

As Pakistan borders Afghanistan, it has made Pakistan a major hub for the transportation, storage, and consumption of heroin and other opiates. Balochistan, Khyber Pakhtunkhwa, and coastal Sindh are major trafficking routes that facilitate the movement of drugs toward international markets through the Arabian Sea and overland routes like the Balkan route (Latif, 2022). A significant amount of this trafficking, leaks into local markets, increasing domestic availability and lowering prices, even though the majority of it is meant for export. This has led to changes in drug consumption patterns over time, such as an increase in injectable drug use and the spread of synthetic drugs like methamphetamine. It has also normalized drug exposure in some communities.

As reported by UNODC in 2017, opium poppy cultivation in Afghanistan expanded sharply, rising by 63% to 328,000 hectares compared to the previous year. Opium production followed a similar trend, increasing by 87% from 4,800 tons in 2016 to 9,000 tons in 2017 (Coomber, 2023). These upward trends have shown no meaningful reversal in subsequent years and have continued to heighten Pakistan's vulnerability to drug trafficking, alongside increasing the risk of greater domestic drug consumption.

Selected Community: Residents of Khyber Pakhtunkhwa (Pathans)

Community Profiling

The selected community are Pathans, residents of Khyber Pakhtunkhwa. As drug addiction is a very common issue in KPK, mostly due to easy accessibility of drugs on account of being proximal to Afghanistan, illiteracy, and lack of implementation of laws to deal with drug trafficking (Khan et al., 2021). According to Drug Use in Pakistan 2013

Summary Report, the highest prevalence of drug use is reported in Khyber Pakhtunkhwa, where 11% of the population use illicit substances (UNODC, 2013).

Age. Studies consistently indicate that young people aged 15–35 years constitute a major subgroup of drug users in Khyber Pakhtunkhwa, with many studies highlighting initiation during the teenage years (Khan et al.,2021). Early exposure is often linked to peer influence, curiosity, academic stress, and easy access to substances, increasing the likelihood of continued use into adulthood and elevating the risk of dependence and associated psychosocial harms.

Gender. It has been found in different studies that men report using drugs more than females (Jan et al., 2021), mainly due to silent epidemic discussed above. A study conducted on 12 women from Peshawar and Mardan, aged 18-21 years, reported using drugs due to peer pressure, emotional challenges and financial concerns.

Socioeconomic Status. Studies demonstrate that drugs users belong from various socioeconomic status, lower to upper class (Fahad et al., 2024).

Substance Used

Cannabis (5.1%), opioids (5.8%), and tranquillizers and sedatives (2.4%) are the most commonly used substances in the province. Around 5.1% of the population in Khyber Pakhtunkhwa uses Cannabis, 5.8% of the population uses opioids and tranquillizers and sedatives are used by 2.4% of the population while opiates are used by around 1.4% of the population. About 30,000 men and women in Khyber Pakhtunkhwa have used amphetamine-type stimulants (ATS), marking the highest prevalence of ATS use in the country. These statistics were reported in Drug Use in Pakistan 2013 Summary Report. These statistics must have surged over the years, as the production of opioids in Afghanistan has considerably increased after 2013.

In Iftikhar Psychiatric Hospital in Peshawar, 0.74% of patients misused cocaine, along with other substances including opiate (84%), marijuana (76%), benzodiazepines (43%), methamphetamines (8%), and tricyclic antidepressants (4%) (Khan et al., 2021). This shows change of drug use patterns from 2013 Drug Use in Pakistan Summary Report.

Research on drug use in women in Khyber Pakhtunkhwa revealed the use of crystal methamphitamine, cocaine, opioids, and heroin (Khan et al., 2025). The sample size was quite limited, but it shows that prevalence estimates for women are likely underestimated due to cultural stereotyping and under reporting (UNODC, 2013).

Cannabis Use and Cultural Acceptance

Cannabis use in Khyber Pakhtunkhwa is considered cultural acceptable, as it grows significantly there. Although cannabis infest many districts of Khyber Pakhtunkhwa province, but more affected districts are Swabi, Mardan, Charsadda and Peshawar where it is highly invasive and invaded most of the open spaces (Khan et al., 2014).

During the fieldwork, patients in Greenfield Hospital of Psychiatry, were mostly from Khyber Pakhtunkhwa. One of them reported, “Cannabis is not a drug, it is just a cigarette.” Similary another one reported in urdu as “ Nashay; na-shay, meaning nothing.” Most of the individuals in rehabilitation use poly drugs, cannabis is considered insignificant and mostly is paired with other drugs like opiods, heroin and methamphetamines.

The relationship between drug use and mysticism has been examined by researchers in various cultural contexts worldwide. In Pakistan, although hashish use is not an official or defining feature of religious shrines, a noticeable number of hashish users are commonly present at Qalandri shrines. Some Malangs reportedly use marijuana as a means of facilitating altered states of consciousness or mystical experiences. At Rahman Baba Shrine in Peshawar, only marijuana is used at the shrine as a holy or acceptable drug. They view substance use as

a source of love for humanity, relaxation from social stresses, and harmony among the users (Ahmed et al., 2022). Over time, this practice has gained a degree of social acceptance within these shrine settings, where it is often viewed as part of a spiritual or devotional tradition rather than as conventional substance use.

Psychosocial Implications

A study identified a significant association between frequent use of ice (methamphetamine) and various negative socio-economic consequences for users. These effects include social stigma, loss of respect, family conflicts, and mutual neglect between addicts and their families. Users were also more likely to face taunting, engage in anti-social or criminal activities to support their drug habit, live in isolation, and experience financial difficulties, sometimes selling valuables or resorting to theft. The study concludes that methamphetamine is a highly harmful drug, with its social and economic impacts often driving users toward criminal behavior and further anti-social actions (Jan et al., 2021).

Treatment and Rehabilitation Resources

Despite millions suffering from drug dependence, governmental and NGO treatment resources are extremely limited. According to Drug Use 2013 Summary Report, in Khyber Pakhtunkhwa total services (structured drug treatment) for drug users are 17, having capacity of 1052 drug users annually (UNODC, 2013). These statistics might have increased over the years, but the underfunded healthcare system might have not allowed for a big change.

According to Drug Use 2013 Summary Report, only 30,000 individuals received structured treatment (UNODC, 2013). While treatment facilities are available in major urban centres such as Lahore and Karachi, their accessibility and quality remain uneven, and many reportedly offer limited psychosocial interventions alongside medical care. Recent literature emphasizes the importance of developing culturally appropriate, gender-responsive

psychosocial services, strengthening community-based outreach, and integrating mental health care within substance use treatment, with particular attention to the needs of women and adolescents (Khan et al., 2025).

Recommendations

Improve National Data Collection on Drug Use

There is a need to complete and formally publish the National Drug Use Survey 2022–24, ensuring that it provides comprehensive and reliable data. The survey should include detailed analyses of drug use trends over time, clear age and gender disaggregation, regional comparisons, and specific attention to emerging substances such as synthetic drugs. Robust reporting in these areas is essential for informed policy-making, targeted prevention strategies, and effective allocation of resources.

Community-Level Prevention

Community-level prevention efforts should prioritize school- and university-based education programs that equip young people with skills to resist peer pressure, manage stress effectively, and adopt healthy alternatives to substance use. Such programs can promote awareness, resilience, and informed decision-making within academic and community setting

Expanded Access to Treatment Services

There is a need to improve both the availability and quality of treatment services for substance use disorders. Expanding biomedical care ensures access to medical detoxification, medication-assisted treatment, and clinical management, while strengthening psychosocial rehabilitation addresses counselling, therapy, relapse prevention, and social reintegration. Special emphasis on rural areas and underserved urban slums is crucial, as these populations often face limited services, financial barriers, and stigma, resulting in delayed or no treatment.

Enhancing access in these settings can help reduce treatment gaps and promote more equitable recovery outcomes.

Gender- Responsive Approaches

Women and girls often face unique challenges in accessing substance use treatment, including heightened stigma, limited autonomy, and underreporting. Gender-responsive and culturally sensitive models can improve engagement, address co-occurring mental health needs, and ensure safer, more accessible pathways to care.

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